

Day/Night Hospitalist Services

Phoenix Indian Medical Center (PIMC)

NOTICE TYPE: Sources Sought / Market Research

Notice ID: SS-26-PHX-018

1. PURPOSE

This is a Sources Sought Notice issued for market research purposes only in accordance with FAR Part 10. This is not a solicitation and does not constitute a commitment by the Government.

The Phoenix Area Indian Health Service (IHS) is conducting market research to determine the availability and capability of:

- Indian Small Business Economic Enterprises (ISBEE)
- Other Small Business Concerns

capable of providing **non-personal physician hospitalist services** for the Phoenix Indian Medical Center (PIMC), Phoenix, Arizona.

The Government intends to use the results of this notice to determine the appropriate acquisition strategy, including potential set-aside determinations under the Buy Indian Act and/or FAR Part 19.

2. REQUIREMENT OVERVIEW

The Government anticipates a contract with a 12-month base year and four (4) 12-month option years.

The Contractor shall provide non-personal hospitalist services, including:

- Inpatient admissions, evaluations, and medical management
- Daily rounding in ICU and medical/surgical units
- Coordination of care, including transfers and discharges
- Timely documentation in accordance with IHS, TJC, and CMS standards
- Collaboration with Government medical staff to support continuity of care

Coverage requirements include:

- **Daytime Shifts:** 7:00 AM – 3:00 PM / 7:00 AM – 6:00 PM
- **Nighttime Shifts:** 6:00 PM – 7:00 AM

Estimated monthly coverage:

- Day shifts: approximately 10–30 shifts
- Night shifts: approximately 15–30 shifts

All services shall be performed on-site at PIMC.

3. IMPORTANT CLARIFICATION

This requirement is for a **non-personal services contract**.

This requirement is NOT for:

- Direct Government supervision of contractor physicians
- Personal services arrangements
- Individual physician hiring or placement by the Government

The Contractor shall be responsible for the management, supervision, and performance of its personnel, while integrating into the clinical environment and coordinating with Government staff.

Responses that demonstrate only individual staffing capability without organizational responsibility for management and performance may be considered non-responsive.

4. TECHNICAL CAPABILITY REQUIREMENTS

Interested firms must demonstrate capability to provide:

A. Staffing and Coverage Model

- Ability to provide continuous inpatient hospitalist coverage
- Staffing model for day and night operations
- Surge and contingency staffing capability

B. Clinical Capability

- Experience providing inpatient hospitalist services
- Ability to manage patients across ICU and medical/surgical units
- Compliance with TJC, CMS, and IHS clinical standards

C. Scheduling and Continuity

- Ability to develop and manage staffing schedules
- Experience maintaining coverage continuity
- Capability to respond to short-notice or emergency staffing needs

D. Credentialing and Qualifications

- Ability to provide licensed physicians meeting IHS credentialing requirements
- Board Certification or Board Eligibility in Internal Medicine or Family Medicine
- BLS and ACLS certification

E. Past Performance

- Experience with contracts of similar size, scope, and complexity
- Experience providing services in facilities comparable to PIMC
- Experience with similar patient volume, acuity, and 24/7 coverage models
- Federal healthcare experience preferred but not required

5. NAICS CODE

NAICS 621111 – Offices of Physicians (except Mental Health Specialists)

Size Standard: \$14.0M

Respondents may recommend an alternative NAICS code with justification.

PSC Code: Q509 - FAMILY & INTERNAL MEDICINE SERVICES

6. BUY INDIAN ACT NOTICE AND DEFINITIONS

The IHS Contracting Officer will give priority to Indian Small Business Economic Enterprises (ISBEE) in accordance with the Buy Indian Act.

This Sources Sought Notice is being issued, in part, to determine whether this requirement may be set aside for ISBEE firms.

Respondents asserting ISBEE status shall:

- Identify ownership and control structure
 - Demonstrate compliance with ISBEE requirements
 - Describe how the firm will perform the primary and vital contract requirements
 - Identify subcontractors and percentage of work to be performed
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7. SUBMISSION INSTRUCTIONS

Responses shall not exceed **7 pages**.

Submit electronically to:

Dekovan Cook
Supervisory Contract Specialist
Dekovan.Cook@ihs.gov

Subject Line: Hospitalist Services Sources Sought – PIMC

Submission Deadline: As stated in SAM.gov

Interested parties are encouraged to review the attached draft Performance Work Statement (PWS) for detailed requirements.

8. DISCLAIMER

This notice is for planning purposes only and does not constitute a solicitation. The Government will not reimburse any costs incurred in responding.

The Government reserves the right to determine the appropriate acquisition strategy based on responses received.

END OF NOTICE